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ELITE RECOVERY

SLEEP PROTOCOL

The variable 90 % of athletes ignore, and the one that decides how much muscle you actually build from everything you **train**.

10-MIN ROUTINE

10 HYGIENE RULES

TROUBLESHOOTING

14-DAY LOG



Five cycles

70 %

OF GH IS RELEASED
WHILE ASLEEP

7-9 h

DAILY TARGET

14

DAYS LOGGED

8

PAGES

SLEEP IS YOUR BEST SUPPLEMENT

Most growth hormone is released during sleep, muscle protein synthesis speeds up, cortisol falls and the nervous system regenerates. Sleeping badly three nights in a row can lower testosterone significantly and send next-day appetite through the roof.

70 %

OF GROWTH HORMONE

-15 %TESTOSTERONE AFTER 3 BAD
NIGHTS**18–20 °C**

IDEAL TEMPERATURE

EXACTLY WHAT YOU LOSE WHEN YOU SLEEP BADLY

CONSEQUENCE	MECHANISM	HOW YOU NOTICE IT
Less muscle gain	Growth hormone from deep sleep falls	Training does not translate into progress
More fat stored	Cortisol rises and insulin sensitivity falls	The waist grows even though the diet has not changed
More hunger	Ghrelin rises and leptin falls	Cravings for ultra-processed food in the afternoon
Less strength	Poorer neural recovery	Your usual loads feel heavy
Higher injury risk	Worse coordination and reaction time	Technical errors you did not use to make

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THE ARCHITECTURE OF YOUR NIGHT

PHASE	DURATION	WHAT HAPPENS IN THE BODY	COST OF LOSING IT
Light sleep (N1-N2)	20-30 min	Wake-to-sleep transition; body temperature drops	Trouble getting into deep sleep
Deep sleep (N3)	60-90 min	Growth hormone and muscle repair	Less muscle gain and more injuries
REM sleep	90-110 min	Memory consolidation and emotional regulation	More stress and worse decisions

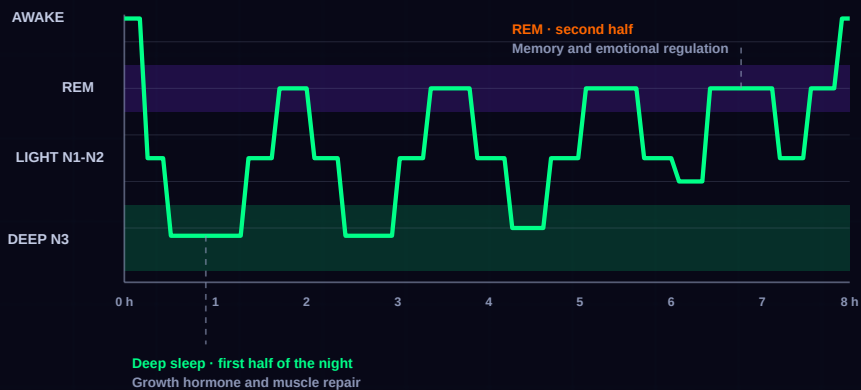
CYCLES LAST ABOUT 90 MINUTES

A full night is 4-6 cycles. That is why waking at 6 h or at 7.5 h feels far better than at 6.5 h: you are cutting the cycle in half, in the middle of deep sleep. Set your bedtime in 90-minute blocks counted back from the alarm.

DEEP SLEEP IS CONCENTRATED AT THE START OF THE NIGHT

- Most **deep sleep** happens in the first two cycles. Going to bed late and compensating by sleeping in does **not** recover that part: it is lost.
- REM** is concentrated in the second half of the night. Cutting the last hour of sleep mainly punishes your emotional regulation and your memory.
- Alcohol brings deep sleep forward but **fragments REM**: you fall asleep sooner and rest far worse.

FIG. 1 ARCHITECTURE OF A FULL NIGHT



Five cycles of about 90 minutes. **Deep sleep** is concentrated at the start and **REM** at the end: that is why going to bed late and getting up late does not recover the same thing.

THE 10-MINUTE ROUTINE

MINUTE	ACTION	TARGET
1-2	Turn off every screen: phone, tablet and television	Remove the blue light that suppresses melatonin
3-4	Darken the room as much as possible	Total darkness maximises melatonin production
5-6	Set the temperature between 18 and 20 °C	Deep sleep requires body temperature to fall
7-8	4-7-8 breathing: four full cycles	Engage the parasympathetic nervous system
9	A quick one-minute body scan	Release the muscular tension built up during the day
10	An intention: «tomorrow I will be better»	Close the day and head off rumination

YOUR IDEAL ENVIRONMENT

VARIABLE	TARGET	HOW TO GET THERE ON A SMALL BUDGET
Light	Total darkness	A cloth eye mask and black tape over device LEDs
Temperature	18-20 °C	Turn the heating down and use breathable bedding
Noise	Constant or absent	Silicone earplugs or white noise (rain, a fan)
Air	Fresh	Air the room for 10 minutes before bed
Mattress and pillow	Neutral neck alignment	A low pillow if you sleep on your front, a high one if on your side
Use of the bed	Sleep only	No work, no series, no phone in the bed

THE LAST HOURS OF THE DAY

HOW LONG BEFORE SLEEP	WHAT TO CUT	WHY
8-10 hours	Caffeine	Its half-life is around 5-6 hours: at 22:00 half a dose is still active
3-4 hours	Intense training	It raises cortisol and body temperature
3 hours	Large meals and alcohol	Digestion and alcohol fragment REM sleep
2 hours	Work and arguments	Mental activation lengthens sleep latency
1 hour	Screens and bright light	It delays melatonin secretion

10 HYGIENE RULES FOR ATHLETES

01 Fixed schedule

Go to bed and get up at the same time all seven days of the week. Your body clock rewards consistency above everything else.

02 A cut-off time for caffeine

No coffee, no green tea and no pre-workout after 14:00. Its half-life means you still have caffeine circulating at midnight.

03 A helpful dinner

Rich in tryptophan — turkey, egg, oats, dairy — with carbohydrate, which helps it reach the brain. Eat 2–3 hours before bed.

04 No alcohol

It fragments REM sleep. A single drink can noticeably reduce the quality of your night even if you fall asleep sooner.

05 Train early

Finish at least 3 hours before bed. Evening training raises cortisol and body temperature exactly when they should be falling.

06 The bed is for sleeping only

No working, no series, no phone in it. The brain learns associations very fast, for better and for worse.

07 A constant ritual

A hot shower, 10 minutes of reading on paper and some breathing. The same sequence every night works as an unmistakable signal for the brain.

08 Total darkness

Use an eye mask if you need to. Even the router's LED can interfere with melatonin production.

09 Silence or constant noise

Rain, a fan or a white noise machine. What wakes you is not the noise itself but the abrupt change in sound level.

10 Morning light

10–15 minutes of natural light when you get up. It is the signal that anchors your circadian rhythm and the reason you will feel sleepy on time at night.

IF YOU CAN ONLY CHANGE ONE THING

Choose the **fixed schedule**. It is the highest-impact intervention and the only one that makes all the others work better. Always getting up at the same time — Sunday included — is worth more than any sleep supplement.

WHEN SOMETHING IS NOT WORKING

YOUR PROBLEM	USUAL CAUSE	WHAT TO DO TONIGHT
It takes me over 30 min to fall asleep	Mental activation and rumination	Get up, go to another room with dim light and come back when you feel sleepy
I wake up at 3 in the morning	Alcohol, a late dinner or stress	No screens and no clock: 4-7-8 breathing lying down, without trying to fall asleep
I sleep but wake up tired	Fragmented sleep or apnoea	Check for snoring and breathing pauses: see a doctor if there are any
I fall asleep watching TV	Poor-quality sleep from light and noise	Move your bedtime 30 min earlier and turn the screen off
I work shifts	Circadian misalignment	Sunglasses on the way home at night, total darkness and a fixed schedule within each shift
I travel and change time zone	Jet lag	Set meals and light to the destination's schedule from day one
I train at night because of work	Late activation	A warm shower on arrival, a light dinner and 10 min of breathing
My phone wakes me	Notifications	Aeroplane mode and charger outside the bedroom, no exceptions

THE NAP, USED WELL

DURATION	EFFECT	RECOMMENDATION
10-20 min	Restores alertness with no sleep inertia	IDEAL before 15:00
30-60 min	You wake up groggy	AVOID IT except in severe deprivation
90 min	A full cycle, no grogginess	VALID only if you slept very little

WHEN TO STOP IMPROVISING AND SEE A DOCTOR

Loud snoring with breathing pauses, disabling daytime sleepiness, insomnia lasting more than three weeks, restless legs at bedtime or waking up choking. These are signs of sleep disorders that require **professional diagnosis**: no hygiene protocol resolves them.

PRE-SLEEP RELAXATION AND LOG

SCRIPT · PROGRESSIVE MUSCLE RELAXATION (5 MIN)

«Lie on your back. Feel the weight of your body on the mattress. Start with the feet: tense the muscles of the foot for 3 seconds and release them completely. Feel the difference between tension and release. Move up through the calves, the thighs, the glutes, the abdomen, the chest, the shoulders, the arms and the neck. When you reach the face, unclench the jaw and soften the eyes. Your whole body is now completely relaxed and heavy. Let sleep come on its own — do not chase it.»

14-NIGHT LOG

NIGHT	BEDTIME	MINUTES TO FALL ASLEEP	AWAKENINGS	WAKE TIME	QUALITY 1-10	10-MIN ROUTINE
1						☐
2						☐
3						☐
4						☐
5						☐
6						☐
7						☐
8						☐
9						☐
10						☐
11						☐
12						☐
13						☐
14						☐

When the two weeks are up, look for the pattern: do your best nights line up with the routine completed, with the fixed schedule, or with having trained in the morning? That is your lever.

WHERE EVERY NUMBER COMES FROM

Every number in this document comes from sources that are public, free and checkable. Not one figure is invented, rounded for convenience or copied from another popular publication. This page exists so that you can verify it yourself.

WHY THESE SOURCES AND NO OTHERS

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WHAT YOU ARE LOOKING FOR	WHAT YOU WILL FIND	WHERE
Reference intakes	RDA, AI and tolerable upper intake level for every vitamin and mineral	ods.od.nih.gov
Food composition	The full profile of any food, nutrient by nutrient	fdc.nal.usda.gov
Dietary guidelines	Dietary patterns and limits for added sugars, saturated fat and sodium	health.gov
Physical activity guidelines	The weekly dose, its response curve and the strength requirement	health.gov
Physical readiness doctrine	FM 7-22: training, nutrition, sleep and mental readiness	armypubs.army.mil
Lifting equation	How much weight is reasonable to lift in each specific situation	cdc.gov/niosh
Food safety	Safe internal temperatures and the danger zone	foodsafety.gov
Health claims	What may legally be claimed about a nutrient in Europe, and what was refused	European Commission register

HOW THIS DOCUMENT CHECKS ITSELF

The reference figures are not typed by hand: they live in a data module that the typesetting program verifies before writing a single line. It checks that the macronutrient ranges can add up to one hundred per cent of energy, that none of the 19 micronutrients has a recommended value above its own ceiling without a note explaining why, and that no safe cooking temperature falls inside the danger zone. If a single check fails, the document is not typeset.

REFERENCE VALUES ARE POPULATION VALUES

They describe what covers the needs of most healthy adults. They are not an individual prescription, they do not account for disease, pregnancy, breastfeeding or medication, and no publication replaces assessment by a registered health professional. If you take medication, some of the nutrients named here interact with it in clinically relevant ways: do not change an intake or start a supplement without telling whoever prescribes for you.

SUPPLEMENTS FOR BETTER SLEEP

None of them replaces sleep hygiene. Use them only once the previous pages are already part of your routine and still fall short.

SUPPLEMENT	DOSE	WHEN	EVIDENCE AND CAVEATS
Magnesium bisglycinate	300–400 mg	30–60 min before	The most useful of the group: it improves depth and helps modulate cortisol
Melatonin	0.5–1 mg	2 h before	It is a timing regulator, not a sleeping pill. Higher doses do not work better
L-theanine	100–200 mg	30 min before	It reduces mental activation without sedating. Good if rumination is your problem
Glycine	3 g	Before sleep	It helps lower core body temperature
Ashwagandha KSM-66	300–600 mg	Night	Moderate evidence on perceived stress and sleep quality

ABOUT MELATONIN

It is not harmless just because it is «natural», and it should not be used daily and indefinitely without supervision. It is not advised for minors, in pregnancy or breastfeeding, or for people with autoimmune diseases or taking anticoagulants or antihypertensives. Check with your doctor or pharmacist before starting.

SLEEP WELL, TRAIN BETTER

Sleep is the front that **decides whether the others pay off**: you train it at the same time every day, measure it over fourteen nights and correct it one variable at a time, exactly like strength.

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